

UUID: E5DD1503-9CED-449F-8312-38D96D9ED951
TCGA-XF-A9SP-01A-PR

Redacted



Collected
Ordered by

FINAL DIAGNOSIS:

RADICAL CYSTOPROSTATECTOMY WITH PELVIC LYMPHADENECTOMY AND CONTINENT URINARY DIVERSION T-POUCH TO URETHRA, G-TUBE PLACEMENT:

RIGHT DISTAL URETER (A):
FROZEN SECTION DIAGNOSIS:
- NO HIGH ATYPIA OR TUMOR IDENTIFIED

FINAL DIAGNOSIS:
- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

C67.9

path

ICD 6-3
Carcinoma, urothelial NOS 8/20/3
Carcinoma, transitional cell NOS 8/20/3

Site: Bladder NOS C67.9

9/3/26/14

LEFT DISTAL URETER (B):
FROZEN SECTION DIAGNOSIS:
- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED
FINAL DIAGNOSIS:
- BENIGN HYDROURETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

PARA-AORTIC LYMPH NODES PROXIMAL LIMITS (C):
FROZEN SECTION DIAGNOSIS:
- BENIGN
FINAL DIAGNOSIS:
- NO METASTATIC CARCINOMA IDENTIFIED IN TWO LYMPH NODES EXAMINED (0/2)

URETHRAL MARGIN (D):
FROZEN SECTION DIAGNOSIS:
- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED
FINAL DIAGNOSIS:
- BENIGN URETHRAL AND PERIURETHRAL TISSUE
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

BLADDER AND PROSTATE (E):

BLADDER:

- IN SITU AND INVASIVE POORLY DIFFERENTIATED TRANSITIONAL CELL CARCINOMA (3.5 X 3.5 X 1.8 CM), GRADE 4/4, INVOLVING TRIGONE AND BLADDER NECK REGIONS AND EXTENDING THROUGH BLADDER WALL AND FOCALLY INTO PERIVESICAL FAT
- LYMPHOVASCULAR SPACE INVASION IDENTIFIED
- REACTIVE CHANGES AND FOCI OF MILD, MODERATE AND SEVERE UROTHELIAL DYSPLASIA AND UROTHELIAL CARCINOMA IN SITU, RANDOM BLADDER
- RESECTION MARGINS, FREE OF UROTHELIAL DYSPLASIA AND MALIGNANCY
- NO LYMPH NODES IDENTIFIED IN PERIVESICAL FAT (0/0)

PROSTATE:

- UNILATERAL/UNIFOCAL MODERATELY DIFFERENTIATED PROSTATIC ADENOCARCINOMA, GLEASON SCORE 6 (3+3), CONFINED TO PROSTATE GLAND WITHOUT CAPSULAR OR PERIPROSTATIC SOFT TISSUE INVOLVEMENT
- ADENOMATOUS HYPERPLASIA WITH FOCI OF HIGH GRADE PROSTATIC INTRAEPITHELIAL NEOPLASIA (PIN II)
- PROSTATIC URETHRA AND PARENCHYMA, NO EVIDENCE OF UROTHELIAL DYSPLASIA OR UROTHELIAL CARCINOMA
- BILATERAL SEMINAL VESICLES, FREE OF TUMOR
- APICAL AND INKED CAPSULAR RESECTION MARGINS, FREE OF UROTHELIAL DYSPLASIA AND MALIGNANCY

PARA-AORTIC LYMPH NODES (F):
- NO METASTATIC CARCINOMA IDENTIFIED IN FOUR LYMPH NODES EXAMINED (0/4)

LEFT COMMON ILIAC LYMPH NODES (G):

REPORT

Collected
Ordered by

- NO METASTATIC CARCINOMA IDENTIFIED IN SEVEN LYMPH NODES EXAMINED (0/7)

PARACAVAL LYMPH NODES (H):

- NO METASTATIC CARCINOMA IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

RIGHT COMMON ILIAC LYMPH NODES (I):

- NO METASTATIC CARCINOMA IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1)

RIGHT EXTERNAL ILIAC LYMPH NODES (J):

- NO METASTATIC CARCINOMA IDENTIFIED IN FOUR LYMPH NODES EXAMINED (0/4)

RIGHT LYMPH NODE OF CLOQUET (K):

- NO METASTATIC CARCINOMA IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1)

RIGHT HYPOGASTRIC / OBTURATOR LYMPH NODES (L):

- NO METASTATIC CARCINOMA IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

LEFT EXTERNAL ILIAC LYMPH NODES (M):

- NO METASTATIC CARCINOMA IDENTIFIED IN FOUR LYMPH NODES EXAMINED (0/4)

LEFT LYMPH NODE OF CLOQUET (N):

- NO METASTATIC CARCINOMA IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1)

LEFT HYPOGASTRIC / OBTURATOR LYMPH NODES (O):

- NO METASTATIC CARCINOMA IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

RIGHT PRESACRAL LYMPH NODES (P):

- NO METASTATIC CARCINOMA IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

PRESACRAL LYMPH NODES (Q):

- NO METASTATIC CARCINOMA IDENTIFIED IN FOUR LYMPH NODES EXAMINED (0/4)

LEFT PRESACRAL LYMPH NODES (R):

- BENIGN FIBROFATTY TISSUE

- NO LYMPHOID TISSUE OR MALIGNANCY IDENTIFIED (0/0) [TOTALLY SUBMITTED]

LEFT PROXIMAL URETER (S):

- BENIGN HYDROURETER

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

RIGHT PROXIMAL URETER (T):

- BENIGN URETER

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

PATHOLOGIC TNM STAGE: BLADDER pT3bN0MX
PROSTATE pT2aN0MX

DIAGNOSIS COMMENT:

The patient's previous biopsy is concurrently reviewed. Representative sections of tumor are submitted for p53 assay by immunohistology, results of which will be reported in and addendum.

INTRAOPERATIVE DIAGNOSIS:

FROZEN SECTION DIAGNOSIS:

AFS: Right distal ureter:

- no high grade atypia or tumor identified

BFS: Left distal ureter:

- no high grade atypia or tumor identified

Collected:
Ordered by:

CFS: Para-aortic lymph nodes proximal limits:
- benign

DFS: Urethral margin:
- no high grade atypia or tumor identified

GROSS DESCRIPTION:

A: The specimen is received fresh from the O.R. and labeled "Right ureter." It consists of a segment of ureter that measures 0.4 cm in length with a diameter of 0.3 cm. Entirely submitted for frozen section in cassette AFS.

B: The specimen is received fresh from the O.R. and labeled "Left ureter." It consists of a dilated ureter that measures 0.6 cm in length with a diameter of 0.4 cm. The specimen is entirely submitted for frozen section.

C: The specimen is received fresh from the O.R. and labeled "Para aortic lymph nodes proximal limits." It consists of a segment of fatty tissue 2.5 x 2 x 1.5 cm containing two grossly identifiable lymph nodes. Lymph nodes are totally submitted for frozen section. The remaining tissue is submitted in cassette C.

D: The specimen is received fresh from the O.R. and labeled "Urethral margin." It consists of a segment of pink-tan, soft tissue measuring 2.5 x 0.4 x 0.2 cm. Entirely submitted for frozen section in cassette DFS.

E: The specimen is received fresh from the O.R. and labeled "Bladder & Prostate." It consists of a cystoprostatectomy specimen which has overall dimensions of 18.5 x 12 x 5 cm. The peritoneum measures 8 x 12 x 0.1 cm and is grossly uninvolved by tumor. The bladder measures 6.5 x 6 x 4 cm. The prostate measures 4 x 4.5 x 3 cm. The specimen is inked and opened anteriorly revealing the prostatic urethra that measures 3 cm in length with a circumference of 1.5 cm. It is lined by pink-tan soft mucosa that is grossly unremarkable. The verumontanum measures 1 x 0.5 x 0.5 cm. There is no excavation of the urethra. On the posterior bladder wall is a raised pale white polypoid tumor mass which measures 3.5 x 3.5 cm and extends to involve the ureterovesical junctions bilaterally. The tumor extends into the trigone and the bladder neck. The remainder of the bladder mucosa is pink-tan, slightly folded and unremarkable. No other tumor or lesions are identified. On sectioning through the tumor, it has a depth of 1.8 cm and grossly appears to invade into the perivesical fat. The right attached ureter measures 5 cm in length with a circumference of 0.8 cm. The mucosa is unremarkable. The left attached ureter measures 0.8 cm in length with a circumference of 1.3 cm and appears to be thickened and dilated. The prostate is serially sectioned. No discrete masses are apparent. Representative sections are submitted in a total of 34 cassettes.

F: The specimen is received in formalin and labeled "Para aortic lymph nodes." It consists of multiple fragments of fatty tissue 3 x 2 x 0.5 cm. Possible lymph nodes are identified and submitted in a single cassette.

G: The specimen is received in formalin and labeled "Left common iliac lymph nodes." It consists of multiple pieces of fatty tissue and lymph nodes 4 x 3 x 1 cm. Multiple lymph nodes are identified, the largest 1.5 cm. The lymph nodes are entirely submitted in four cassettes.

H: The specimen is received in formalin and labeled "Para caval lymph nodes." It consists of a 2 x 2 x 0.5 cm portion of fatty tissue with attached lymph node. The lymph node is bisected and entirely submitted in a single cassette.

I: The specimen is received in formalin and labeled "Right common iliac lymph

Collected

Ordered by

nodes." It consists of a 3 x 3 x 1cm portion of fatty tissue. Possible lymph nodes are entirely submitted in a single cassette.

J: The specimen is received in formalin and labeled "Right external iliac lymph nodes." It consists of multiple pieces of fatty tissue with attached lymph nodes 6 x 2 x 2 cm. The largest lymph node measures 1.8 cm. The lymph nodes are entirely submitted in two cassettes.

K: The specimen is received in formalin and labeled "Right lymph node of cloquet." It consists of multiple fragments of fatty tissue that aggregate to 1 x 0.5 x 0.5 cm. A single lymph node is identified and entirely submitted in a single cassette.

L: The specimen is received in formalin and labeled "Right hypogastric / obturator lymph nodes." It consists of a 4 x 3.5 x 1.5 cm aggregate of multiple fatty tissue fragments. Possible lymph nodes are submitted in two cassettes.

M: The specimen is received in formalin and labeled "Left external iliac lymph nodes." It consists of multiple fragments of fatty tissue with possible lymph nodes aggregating to 1 x 2.5 x 0.5 cm. Possible lymph nodes are submitted in a single cassette.

N: The specimen is received in formalin and labeled "Left lymph node of cloquet." It consists of multiple pieces of fatty tissue 2.5 x 2.0 x 1.0 cm in aggregate. No lymph nodes are grossly identified. The specimen is entirely submitted in a single cassette.

O: The specimen is received in formalin and labeled "Left hypogastric obturator lymph nodes." It consists of multiple pieces of fatty tissue with attached lymph nodes that aggregate to 6 x 3.5 x 2.5 cm. The lymph nodes are entirely submitted in three cassettes.

P: The specimen is received in formalin and labeled "Right presciatic lymph nodes." It consists of multiple pieces of pale yellow fatty tissue with attached lymph nodes 2 x 2 x 2 cm. Three lymph nodes are identified and entirely submitted in a single cassette.

Q: The specimen is received in formalin and labeled "Presacral lymph nodes." It consists of multiple pieces of fatty tissue with attached lymph nodes 5 x 4.5 x 4 cm. Three lymph nodes are identified and entirely submitted in a single cassette.

R: The specimen is received in formalin and labeled "Left presciatic lymph nodes." It consists of multiple pieces of fatty tissue with lymph nodes 1.5 x 1 x 1 cm in aggregate. Multiple lymph nodes are entirely submitted in a single cassette.

S: The specimen is received in formalin and labeled "Left proximal ureter." It consists of a segment of ureter that measures 1.5 cm in length. It is thickened and has a diameter of 0.5 cm. The specimen is entirely submitted in a single cassette.

T: The specimen is received in formalin and labeled "Right proximal ureter." It consists of a segment of ureter that measures 1 cm in length with a diameter of 0.4 cm. Entirely submitted in a single cassette.

SECTIONS:

AFS:	frozen section, right ureter
BFS:	frozen section, left ureter
CFS:	frozen section, para-aortic lymph nodes distal limits
CRT:	remaining tissue
DFS:	frozen section, urethral margin
E1:	anterior bladder neck

Transvaginal Cervix

Collected

Ordered by

Location

E2: anterior bladder wall
 E3: right ureterovesical junction
 E4: right lateral wall
 E5: posterior wall
 E6: dome
 E7: left lateral wall
 E8: left ureterovesical junction
 E9: tumor with left anterolateral margin
 E10: tumor, left posterolateral margin
 E11: tumor with left seminal vesicle
 E12: tumor posterior bladder neck and trigone
 E13,14: tumor
 E15: right distal prostate
 E16: left distal prostate
 E17: right midprostate
 E18: left midprostate
 E19,23: right mid anterior prostate
 E20: right mid posterior prostate
 E21,24: left mid anterior prostate
 E22: left mid posterior prostate
 E25: right proximal prostate
 E26: left proximal prostate
 E27: right bladder neck
 E28: left bladder neck
 E29: right bladder neck
 E30: right proximal posterior prostate with seminal vesicle
 E31: left bladder neck
 E32: left proximal posterior prostate with seminal vesicle
 E33: right perivesical fat, possible lymph node
 E34: left perivesical fat, possible lymph node
 F: para-aortic lymph nodes
 G1-4: left common iliac nodes, each cassette, bisected lymph node
 H: paracaval lymph nodes
 I: right common iliac lymph nodes
 J1: right external iliac, single lymph node, bisected
 J2: two lymph nodes
 K: right lymph node of Cloquet
 L1,2: right hypogastric/obturator lymph nodes
 M: left external iliac lymph nodes
 N: left lymph node of Cloquet
 O1-3: left hypogastric obturator, each lymph node, bisected
 P: right presciatic lymph nodes
 Q: presacral lymph nodes
 R: left presciatic lymph nodes (entirely submitted)
 S: left proximal ureter
 T: right proximal ureter

MICROSCOPIC DESCRIPTION:

A-D: See final microscopic-diagnosis.

D: Sections corresponding to the tumor in the posterior bladder wall show a deeply invasive poorly differentiated transitional cell carcinoma of the bladder measuring 3.5 x 3.5 x 1.8 cm. The tumor infiltrates the lamina propria and muscularis propria and into the superficial perivesical fat. The neoplastic cells invade as nests, cords, sheets and as single cells. They have high nuclear to cytoplasmic ratios, hyperchromatic pleomorphic nuclei and prominent nucleoli. Multifocal lymphovascular invasion identified. The tumor involves the posterior wall and extends into the trigone and bladder neck distally and extends bilaterally to

Temporary Copy

Collected

Ordered by

Location

involve the ureterovesical junctions (E3,8-14, 27-29). Multifocal urothelial carcinoma in situ is also noted (E1-7) with invasion of the lamina propria (E3,4,7). Sections of the tumor adjacent to the left seminal vesicle shows tumor in the perivesical fat but not involving the seminal vesicle. Sections that include the bladder neck show invasive tumor extending near but not involve prostatic tissue. Resection margins are free of urothelial dysplasia and malignancy.

Sections of the prostate show a moderately differentiated adenocarcinoma (E19), involving the right mid anterior prostate, Gleason's score 6 (3+3). The tumor is characterized by single variably shaped and sized malignant glands that infiltrate the stroma in a random fashion. The neoplastic cells are medium to large with high nuclear to cytoplasmic ratios, vesicular chromatin and prominent nucleoli. The prostatic capsule, periprostatic soft tissue and inked capsular margins are free of tumor. The bilateral seminal vesicles are free of tumor. No perineural invasion is identified. The non-neoplastic prostate shows benign prostatic hyperplasia with rare foci of high grade prostatic intraepithelial neoplasia (PIN II). The prostatic urethra is free of urothelial dysplasia and the prostatic itself shows no evidence of invasive urothelial carcinoma.

F-T: See final microscopic-diagnosis.

Signed by:

ELECTRONIC SIGNATURE

POST-OPERATIVE DIAGNOSIS:

Same as preop

PRE-OPERATIVE DIAGNOSIS:

Muscle invasive bladder TCC

ORDERING PHYSICIAN:

Ordering Physician:

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted	☒	
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 1/3/2014	